

BriaCell Bria-ABC Phase 3: 144-Death Event-Accrual Monte Carlo Report

Prepared: 28 June 2026 · Model as-of date: 26 June 2026 · Language: English · Purpose: clinical event-timing analysis; not investment advice, medical advice, or a regulatory prediction.

Executive summary

This report constructs an enrollment-adjusted Monte Carlo event-accrual model to estimate when BriaCell's Bria-ABC Phase 3 trial may reach the interim threshold of **144 patient events, defined as deaths**. The model uses reconstructed patient entry dates, accelerating enrollment assumptions, and arm-specific overall-survival distributions.

Main result: under the base accelerating enrollment assumption, if Bria-IMT+CPI is inactive and both main arms behave like standard therapy, the median date of the 144th death is **05 Dec 2026**. Under a Phase 1/2-consistent Bria survival-effect scenario, the median date shifts to **11 Feb 2027**. The difference is approximately **68 days**, or about **2.2 months**.

The limited size of this calendar shift is expected: the 144 events do not come only from the active treatment arm. They accrue across the Bria-IMT+CPI and physician's choice/TPC arms combined, while the TPC arm continues to generate deaths relatively quickly.

>230 total

Last official enrollment point: 12 May 2026; modeled conservatively as 230 total enrolled.

202 patients

Modeled main analysis set as of 26 Jun 2026, excluding monotherapy.

354 patients

Target main analysis set: 404 total minus approximately 50 monotherapy patients.

1. Source data and interpretation of the trial design

BriaCell reported on 12 May 2026 that more than 315 patients had been screened and more than 230 patients had been enrolled in the Bria-ABC Phase 3 study. The same source language frames the pivotal comparison as Bria-IMT plus a checkpoint inhibitor versus physician's choice/TPC, with an interim analysis after **144 patient events - deaths** and overall survival as the primary endpoint.

The ClinicalTrials.gov record describes the study as Bria-IMT regimen plus checkpoint inhibitor/retifanlimab versus treatment of patients'/physicians' choice. Because the monotherapy arm is discontinued after the first 150 patients, the base model excludes the approximately 50 Bria-IMT monotherapy patients from the 144-event main analysis set. This is the interpretation most consistent with public source language; however, because the full statistical analysis plan is not public, an all-randomized sensitivity analysis should be retained as a robustness layer.

Enrollment chronology and conversion into the main analysis set

Date	Official / derived information	Modeled main-analysis patients	Comment
Jan 2024	Assumed enrollment start	0	Starting anchor
22 Apr 2025	>75 total enrolled	~50	Roughly two-thirds of the first 150 belong to the two main arms
09 Dec 2025	>160 total enrolled	~110	The 150-patient threshold is assumed to have been crossed; ~50 monotherapy patients excluded
12 May 2026	>230 total enrolled	180	230 total minus ~50 monotherapy patients
26 Jun 2026	No new official data	202	+22 main-analysis patients estimated after May 12 using accelerating enrollment

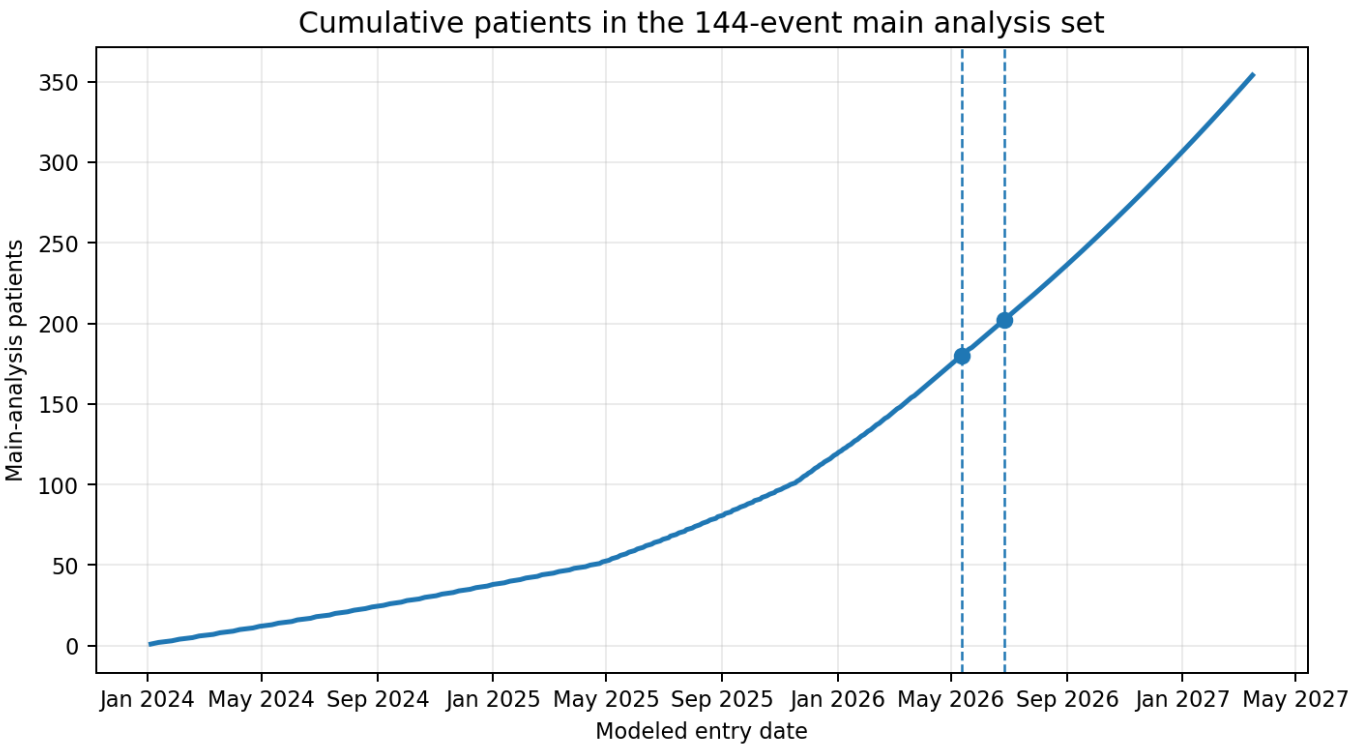


Figure 1. Cumulative patient entries into the main 144-event analysis set. The post-12 May segment is model-derived rather than officially reported.

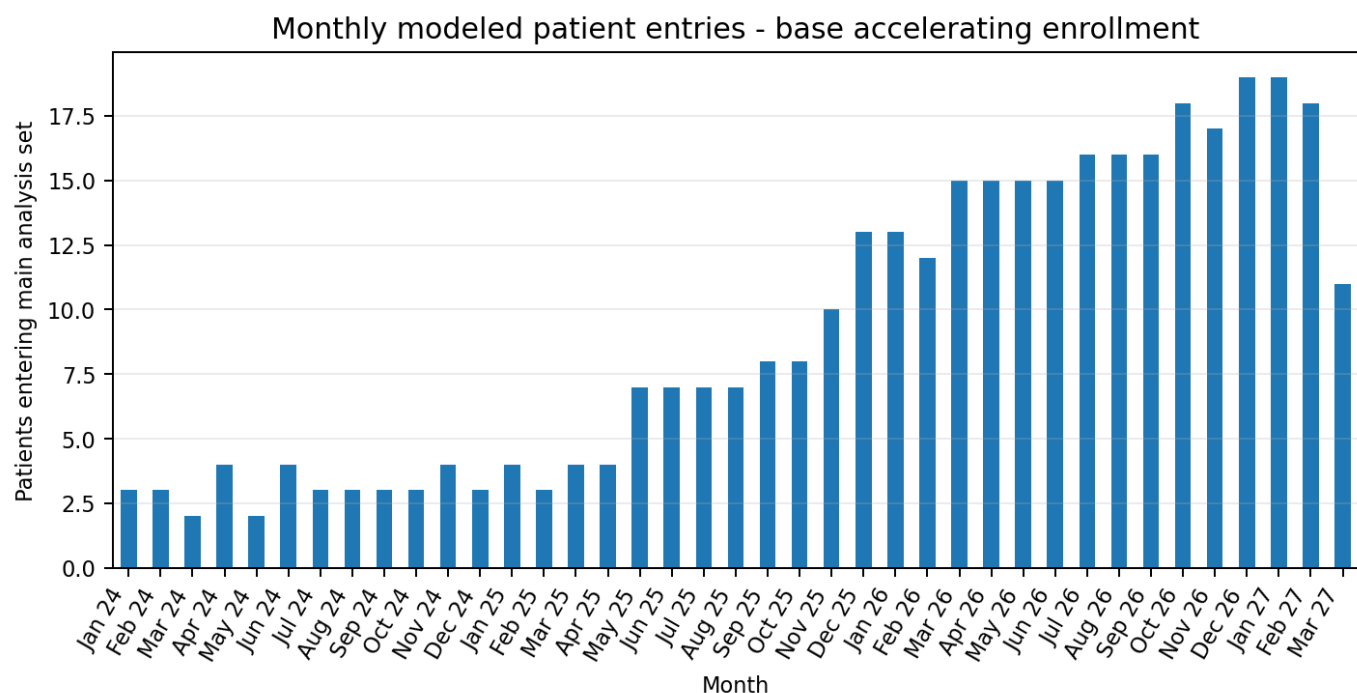


Figure 2. Monthly patient entries in the base accelerating enrollment scenario. The model explicitly includes a later-period acceleration/tail effect.

2. Enrollment assumptions

After May 2026, three dynamic enrollment scenarios are used to represent continued acceleration. Since the monotherapy arm is assumed to be closed after patient 150, every new post-threshold patient enters the main two-arm analysis set.

Scenario	Initial rate	Monthly increase	Rate cap	Average future rate	Main-set completion
Conservative acceleration	14/mo	+0.4/mo	20/mo	16.0/mo	12 Apr 2027
Base acceleration	15/mo	+0.6/mo	22/mo	17.6/mo	16 Mar 2027
Fast acceleration	16/mo	+0.9/mo	26/mo	19.5/mo	18 Feb 2027

3. Survival distributions

The simulation uses two main families of survival assumptions:

- **HR=1 / inactive-drug scenario:** Bria-IMT+CPI and TPC share the same standard-therapy survival distribution. Median OS sub-scenarios are 7, 9, and 11 months.
- **Phase 1/2-consistent Bria-effect scenario:** the TPC control arm is held around 9-11 months, while the Bria-IMT+CPI arm is assigned median OS values in the 14-17.3 month range derived from BriaCell's Phase 1/2 survival disclosures.

The main survival distribution is exponential: $\lambda = \ln(2) / \text{median OS}$. This is a transparent starting assumption for event-accrual modeling when public data provide median OS estimates but not

patient-level Kaplan-Meier curves. A future robustness layer should add Weibull shape sensitivities at 0.85, 1.00, and 1.15.

4. Monte Carlo simulation design

- Each patient's `entry_date` is fixed and comes from the reconstructed enrollment schedule.
- Each patient is assigned to Bria-IMT+CPI or TPC using balanced 1:1 allocation.
- Each patient receives a scenario-specific median OS and monthly hazard.
- Survival time is drawn from the exponential distribution, and `death_date = entry_date + survival time`.
- Death dates are sorted, and the 144th death date is stored in each iteration.
- Each scenario is run with **50,000 Monte Carlo replications**.

5. Main results: base accelerating enrollment

Scenario	Bria OS	TPC OS	Median 144th death	90% interval	P(144 by 31 Dec 2026)	P(144 by 31 Mar 2027)
HR=1 fast standard (7/7 mo)	7 mo	7 mo	17 Oct 2026	19 Sep 2026 - 15 Nov 2026	100.0%	100.0%
HR=1 base inactive (9/9 mo)	9 mo	9 mo	05 Dec 2026	04 Nov 2026 - 07 Jan 2027	91.3%	100.0%
HR=1 slow standard (11/11 mo)	11 mo	11 mo	19 Jan 2027	15 Dec 2026 - 23 Feb 2027	19.7%	100.0%
Phase 1/2 conservative (14/9 mo)	14 mo	9 mo	23 Jan 2027	19 Dec 2026 - 28 Feb 2027	14.6%	99.9%
Phase 1/2 base (16.5/9 mo)	16.5 mo	9 mo	11 Feb 2027	06 Jan 2027 - 19 Mar 2027	3.0%	98.5%
Phase 1/2 optimistic (17.3/11 mo)	17.3 mo	11 mo	15 Mar 2027	05 Feb 2027 - 25 Apr 2027	0.1%	75.7%

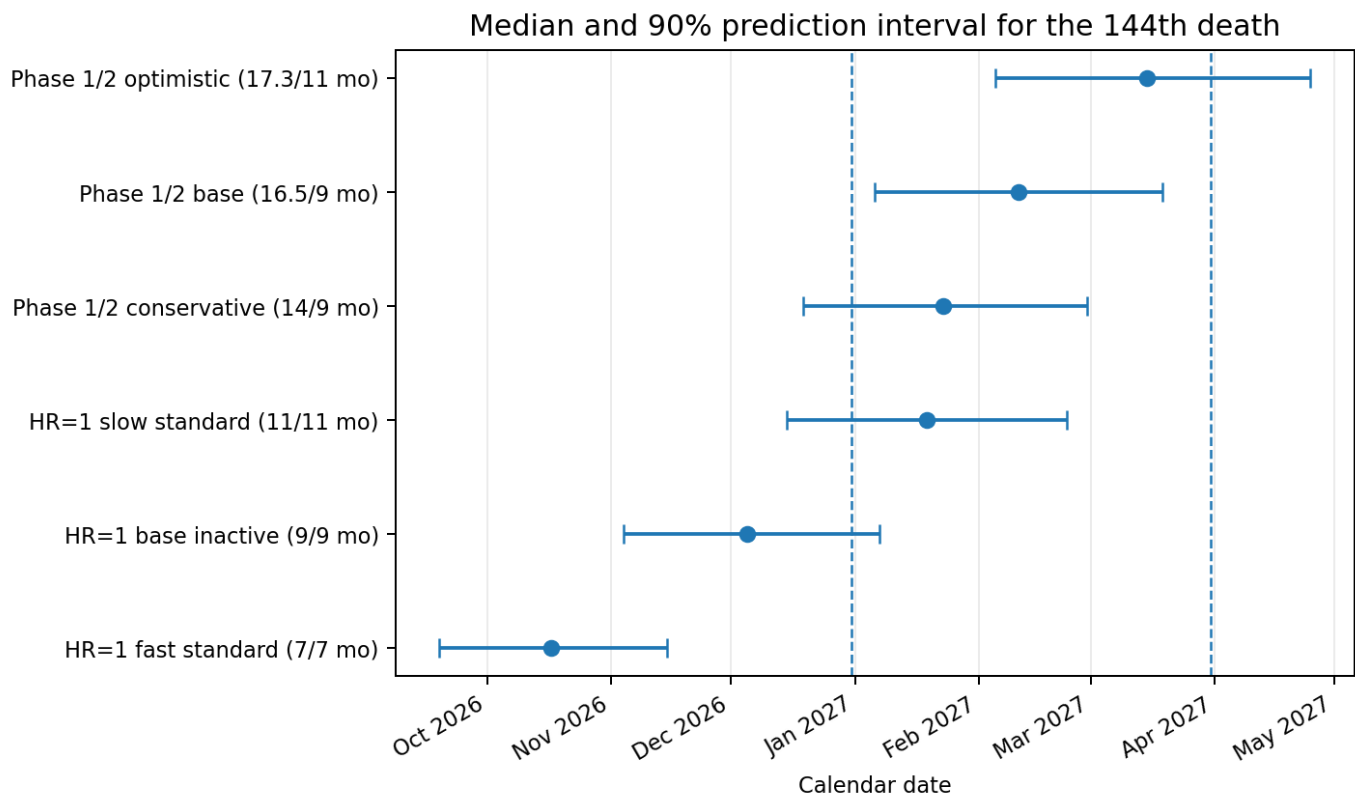


Figure 3. Median 144th-death date and 90% prediction interval under base accelerating enrollment.

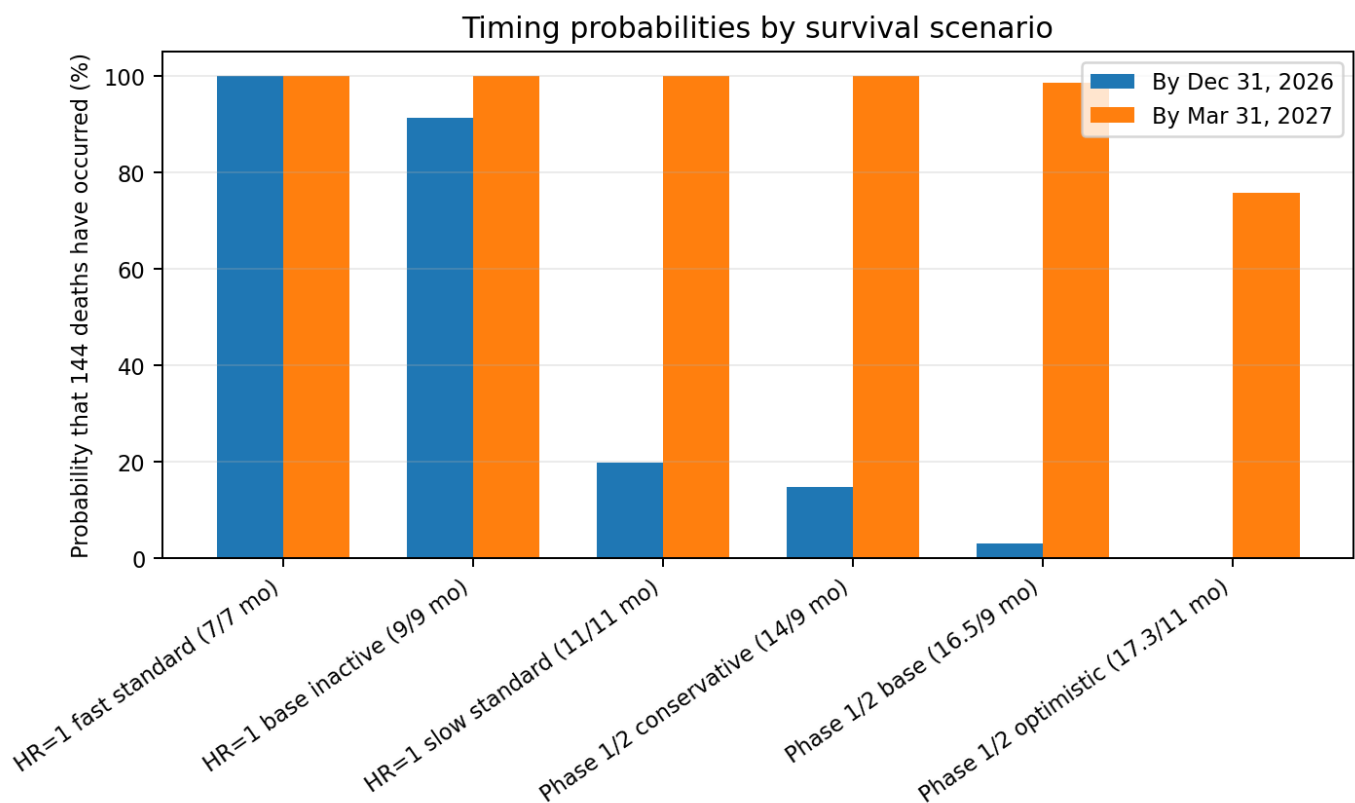


Figure 4. Probability of reaching 144 deaths by selected calendar cutoffs.

6. Enrollment-acceleration sensitivity

Continued acceleration in patient enrollment pulls the 144th-death date forward, but only modestly. Newly enrolled patients expand the risk set, but they also require follow-up time before producing events. Therefore, accelerating enrollment keeps the HR=1 base scenario centered around December 2026, while the Phase 1/2-consistent Bria-effect scenario remains centered around February 2027.

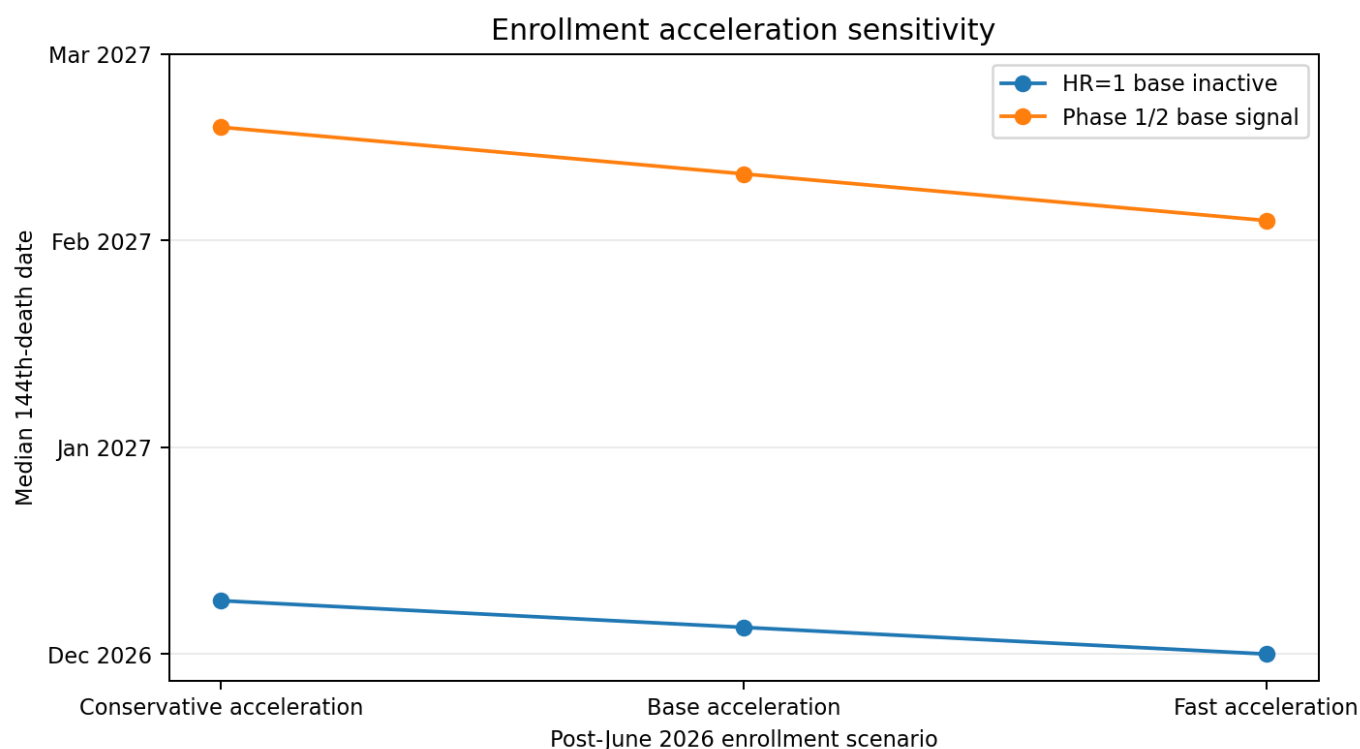


Figure 5. Median 144th-death dates under accelerating enrollment scenarios for the two core survival cases.

7. Clinical and analytical interpretation

If the drug is inactive: in the base accelerating enrollment and HR=1 9/9-month scenario, the median date for the 144th death is **05 Dec 2026**. This can be used as an inactive-drug event-accrual reference line.

If the Phase 1/2 signal is real: when the Bria-IMT+CPI arm is modeled at 16.5 months median OS and the TPC arm at 9 months, the median 144th-death date becomes **11 Feb 2027**. This is an approximately **2.2-month delay**.

This delay may look small, but it is consistent with event-driven trial mechanics. The 144 deaths accrue across both main arms, not only from the Bria arm. Because the TPC arm continues to generate events quickly, a meaningful survival extension in the active arm translates into a limited but potentially informative calendar delay.

Interpretation boundary: a delay beyond the HR=1 event-accrual reference is not, by itself, proof of efficacy. However, if enrollment, analysis-set, and reporting-lag assumptions are

approximately correct, a sustained delay would be directionally consistent with longer-than-expected survival in the Bria-IMT+CPI arm.

8. Financial overlay: cash burn, runway, and dilution risk

The event-accrual model should not be interpreted in isolation. For a clinical-stage biotech, a later 144-event date may be directionally more consistent with longer survival in the active arm, but it also consumes more cash and can increase financing pressure before the information window is reached.

As of 30 Apr 2026, BriaCell reported approximately **\$6.9M** in cash and cash equivalents and **\$15.9M** in short-term investments, or about **\$22.8M** of cash-like liquidity. Net cash used in operating activities was approximately **\$22.7M** over the nine months ended 30 Apr 2026, implying a historical average operating cash burn of roughly **\$2.5M per month**. In early June 2026, the company also closed a best-efforts offering of **1,449,300 common shares** at **\$3.25** per share, for approximately **\$4.7M** in gross proceeds. For the matrix below, the working assumption is an effective liquidity level of **\$22.5M as of 26 Jun 2026**, after allowing for May-June burn and the June offering.

Important financing point: accelerating enrollment is not a free positive. It may bring the clinical information window closer, but it can also increase trial-related cash burn through higher site activity, screening, treatment initiation, manufacturing, CRO monitoring, data management, safety follow-up, and biomarker/imaging costs.

8.1 Cash remaining at modeled 144th-event dates

Clinical scenario	Median 144th-event date	Months from 26 Jun 2026	\$2.5M/mo flat burn	\$3.0M/mo enrollment-adjusted	\$3.5M/mo high-intensity
HR=1 fast event (7/7 mo)	17 Oct 2026	3.7	\$13.2M	\$11.4M	\$9.5M
HR=1 base inactive (9/9 mo)	05 Dec 2026	5.3	\$9.2M	\$6.5M	\$3.9M
HR=1 slow standard (11/11 mo)	19 Jan 2027	6.8	\$5.5M	\$2.1M	\$-1.3M
Phase 1/2 conservative (14/9 mo)	23 Jan 2027	6.9	\$5.2M	\$1.7M	\$-1.8M
Phase 1/2 base (16.5/9 mo)	11 Feb 2027	7.6	\$3.6M	\$-0.2M	\$-3.9M
Phase 1/2 optimistic (17.3/11 mo)	15 Mar 2027	8.6	\$1.0M	\$-3.3M	\$-7.6M

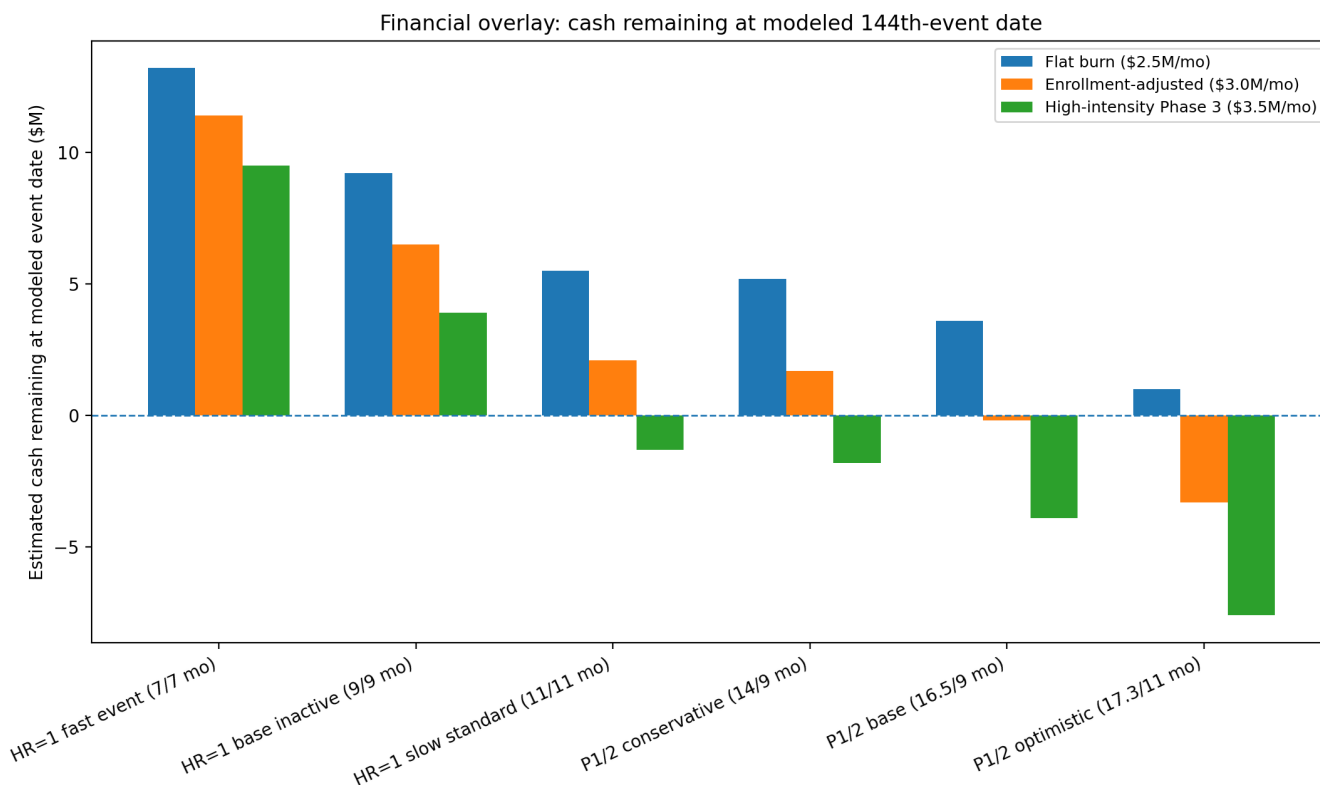


Figure 6. Estimated cash remaining at each modeled 144th-event date under three monthly burn assumptions. Negative values indicate a likely financing need before that event date, absent additional capital or lower actual burn.

8.2 Investment-relevant interpretation

The clinical and financing models work in opposite directions. A delayed 144-event date may become more encouraging clinically if enrollment assumptions remain reasonable, but the same delay reduces the cash cushion. Under a flat burn assumption of approximately **\$2.5M/month**, the company may be able to reach the February-March 2027 information window, although with limited cushion. Under enrollment-adjusted or high-intensity Phase 3 burn assumptions of **\$3.0M-\$3.5M/month**, the runway becomes materially tighter and the probability of pre-readout or near-readout dilution increases.

Therefore, the key equity-research question is not only whether the event clock is moving later than the HR=1 reference line. It is whether the company can finance itself to that later information window without materially diluting shareholders.

Financial sources: BriaCell Form 10-Q for the quarter ended 30 Apr 2026; BriaCell June 2026 offering disclosure. This section is not a valuation, price target, or investment recommendation.

9. Limitations and recommended robustness layer

- **No full SAP/protocol:** excluding monotherapy is based on public source language; an all-randomized sensitivity analysis should be added.
- **"Over 230" uncertainty:** 230 is treated as a conservative lower bound; alternative May 2026 baselines such as 240 or 250 should be tested.
- **No patient-level profile data:** subtype, ECOG status, age, HLA matching, and prior-line counts are not known at patient level; survival is modeled by arm and scenario.

- **Distribution sensitivity:** exponential is the main model; Weibull shapes of 0.85 and 1.15 should test early-risk and rising-risk alternatives.
- **Announcement lag:** the simulation estimates the true date on which the 144th death occurs. Public topline disclosure may lag by 30-60 days or more due to data cutoff, validation, database lock, DSMB review, and analysis.

10. Sources

1. BriaCell, "Phase 3 Bria-IMT Study Enrollment Surpasses 230 Patients", 12 May 2026. <https://briacell.com/briacell-phase-3-bria-imttm-study-enrollment-surpasses-230-patients/>
2. ClinicalTrials.gov, NCT06072612, "Study of the Bria-IMT Regimen and CPI vs Physicians' Choice in Advanced Metastatic Breast Cancer". <https://clinicaltrials.gov/study/NCT06072612>
3. BriaCell, "Phase 3 Patient Enrollment on Track for 1H2026 Topline Data Readout", 9 Dec 2025. <https://briacell.com/briacells-phase-3-patient-enrollment-on-track-for-1h2026-topline-data-readout/>
4. BriaCell, "Reports Robust Overall Survival and Clinical Benefit Data at ASCO 2025", 2 Jun 2025. <https://briacell.com/briacell-reports-robust-overall-survival-and-clinical-benefit-data-at-asco-2025/>
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6. BriaCell Form 10-Q for quarter ended 30 Apr 2026. <https://www.sec.gov/Archives/edgar/data/1610820/000149315226027928/form10-q.htm>
7. BriaCell Therapeutics Announces Closing of Offering, 2 Jun 2026. <https://www.globenewswire.com/news-release/2026/06/02/3092818/0/en/BriaCell-Therapeutics-Announces-Closing-of-Offering.html>

Important note: This report is for modeling and research purposes only. It should not be used as investment advice, medical advice, or a regulatory-outcome prediction. All estimates are conditional on public information and explicitly stated assumptions.